

إسم المريض رباعياً : نجلاء احمد السيد احمد
المحافظة : القاهرة الحي / المدينة : دار السلام
رقم البطاقة : 28211280104961 رقم التليفون : 01099416553
العمر(تاريخ الميلاد) : 28-11-1982 المهنة : ربه منزل
العنوان : دار السلام
تاريخ تسجيل الإستمارة : 01-10-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: نجلاء احمد السيد احمد National ID: 28211280104961	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 نجلاء
 احمد السيد احمد خورشيد
 ٧ شن هاشم العطار من القيوم
 دار السلام - القاهرة
 ٢٨٢١١٢٨٠١٠٤٩٩١
 ٢٨٢١١٢٨٠١٠٤٩٩١
 LA7753247


 ٢٠٢٥/٠٣
 ٢٨٢١١٢٨٠١٠٤٩٩١
 حصله على دبلوم صنايع
 منشئة
 أنشئ
 ٢٠٢٢/٠٢/٠٨
 البطاقة مارية حتى


١٤



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي للصحة النسائية
وزارة الصحة والسكان



A-Patient ID:

1- Name: نجلاء أحمد السيد أحمد

Sex:

MDT form

3- Medical number:

Female

83159859

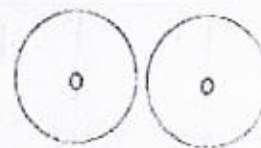
Old Med:

Old Med:

3228-2023

2-ID number: 28211280104961

Patient Age: 41 Y



Family history

Family history

Phone

Phone

01028223221

BMI

BMI

REASON OF MDT

REASON OF MDT
Most frequent

Marital status

Marital status
Married

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

23/03/2024

Co morbidity

Co morbidity
+ve

Menopausal
status

Menopausal status
Menopausal

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

LT Breast mass + LT PLN

RT SOLITARY retro areolar ducts

Mets Workup :

CT CAP : Free of mets

BR IDC	ER 8/8	PR 6/8	Her 2 Neu 0	SISH	Ki67 30
BR CONFIDENT	ER CONFIDENT	PR CONFIDENT	Her 2 Neu CONFIDENT	SISH CONFIDENT	Ki67 CONFIDENT

T 4d	N 3	M 0
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Sonomammography

Lt breast mass + ipsilateral path LNs

PATIENT HISTORY

known case of LT LABC

received AC x4 + 12 wks taxol with good clinical response

It modified radical mastectomy: multicentric IDC GII with focal ductal carcinoma in situ

free resection margins

free skin

LN +ve metastatic deposits 9/12 with evidence of capsular invasion

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

PORT,, AI zoladex for 5- 7 years

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY Signature: Sh Ha	RADIOLOGY	RADIOLOGY Signature: Dr. H. H. H.
SURGERY	SURGERY Signature: Dr. H. H. H.	PATHOLOGY	PATHOLOGY Signature: Dr. H. H. H.
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: نجلاء احمد السيد			
MR. : 6725			
Detailed Medical Report			
		السيد	
تحيه طيبة وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريرنا عن حالتها			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها			
A CASE OF BC ON HT AI ZOLADEX TODAY FOR NEW KARAR AND MEDICAL REPORT DATE OF NEXT VISIT CLINIC AFTER 1 MONTH			
4/2024 mdt : known case of LT LABC received AC x4 + 12 wks taxol with good clinical response It modified radical mastectomy:multicentric IDC GII with focal ductal carcinoma in situ free resection margins free skin LN -Five metastatic deposits 9/12 with evidence of capsular invasion 1/porth 2/ai +zoladex 7 years aramidex +zoladex +supportive*3m next visit :19/10/2026			
تحريرا في ,			
التاريخ :		19-09-2026	
الوقت :		nada mohamed elsayed	
الطبيب المعالج :			
يعتمد ,			

Patient Name : الأستاذة / د. أمجد السيد خورشيد
Patient No. : 28211280104961
Age : 1132/24
Sex : Female
Age : 40 Y

Referring Doc :

Request No. : 13006201
Request Date : 23/03/2024
Verification Date : 30/03/2024
Financial Category : نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Medical Data: Left breast cancer, post NACT.

Specimen: Left modified radical mastectomy specimen.

Gross Examination:

Procedure: Left MRM measured 30x23x7 cm, with covering skin ellipse measured 26x16 cm.

Tumor extent: Two greyish white tumor bed sites, located 3 cm apart.

Size: 7x4.5x3.5 cm & 1x1x0.5 cm, located in the central and UOQ respectively.

Additional findings: Multiple satellites beaded grey white areas detected adjacent to the UOQ mass, measured up to 3x2 cm.

Skin: Nipple & areola looked unremarkable grossly.

Distance from margins: 0.5 cm away from the least deep margin.

Lymph nodes number and size: 12 L.Ns, up to 4x3.5x1 cm.

3 Blocks:

2- UOQ TBS. 3-10- Central TBS. 11&12- Satellites adjacent to the UOQ mass. 13- Area between the UOQ & central tumor bed sites. 14- Nipple & areola. 15- Least deep margin.

18- Lymph nodes.

Gross was done by: Dr. Duaa Mohamed.

Microscopic Examination:

Histological type: Viable multicentric invasive duct carcinoma in both UOQ and central areas.

Grade (differentiation+ nuclear pleomorphism+ mitosis): (2+2+2) Grade II.

Maximal tumor diameter 7 cm.

Estimated tumor cellularity: 50%.

In situ carcinoma: Focally detected.

Free.

Verified by :

Dr. Duaa Mohamed

Date : 06/04/2024 12:19:54

Page 1 of 3

Printing By : Mrs Mayada Hegazy

شرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / نجلاء احمد السيد
Patient No. : 28211280104961
Path No. : 2606/23
Sex : Female
Age : 40 Y
Referring Doc. :

Request No. : 13007589
Request Date : 12/08/2023
Verification Date : 24/08/2023
Financial Category : ورام دار السلام - ميلاده
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at .

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
1: (Positive control showed appropriate reactivity):
2: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
3: Intermediate nuclear reaction in 35% tumor cells (Allred score 6/8).
4: No membrane reaction in tumor cells (score 0).
5: 30% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal micropapillary features

ER: Positive (score 8/8).
PR: Positive (score 6/8).
Her2: Negative (score 0).
Ki67: Positive in 30% of tumor cells.

بربابه حسن
٨١٢٧

Signature

Verified by :
Dr. Heba Hasan

g Date : 26/08/2023 10:51:58

Page 1 of 2

Printed By : marwa mohamed yousse

Patient Name : الأستاذة / نجلاء أحمد السيد خورشيد
Patient No. : 28211280104961
Path No. : 1132/24
Sex : Female
Age : 40 Y
Referring Doc :

Request No. : 13006201
Request Date : 23/03/2024
Verification Date : 30/03/2024
Financial Category : نقية الدولة
Department : Out Patient
Registered by : marwa mohamed wanas

Pathology Report

Lymph node involvement: Positive metastatic deposits in nine out of twelve lymph nodes, N (9/12), with evidence of capsular invasion.
Resection Margins: Free.
Histologic response to therapy: Partial pathological response to therapy.
Additional pathologic findings: Adjacent breast tissue showing areas of lympho-vascular invasion & satellites areas.

Diagnosis:
Left breast cancer post NACT, Left modified radical mastectomy specimen:
Multicentric invasive duct carcinoma, grade II, with focal ductal carcinoma in situ.
Maximal tumor diameter: 7 cm.
Actual tumor cellularity: 50%.
Resection Margins: Free.
Lymph nodes: Positive metastatic deposits in nine lymph nodes, L.N (9/12), with evidence of capsular invasion.
Histologic response to therapy: Partial pathological response to therapy.
Stage: ypT3, ypN2a.
S: 4.14, RCB class III.

Signature
Prof. Dr. Iman Gouda
Dr. Heba Hassan

Verified by :
Dr. Duaa Mohamed

Date : 06/04/2024 12:19:54

Page 2 of 2

Printing By : Mrs. Mayada Hingoud

عنوان : رقم (4) - الدور الأول - مبنى رقم (987) - أول نيشن النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Nile El Qadima - Cairo, Egypt

Verified by :
Dr. Heba Hasen

C. Signature

Conclusion:
1 breast cancer, Tru-cut biopsy:
Positive (score 8/8).
Positive (score 6/8).
Negative (score 0).
Positive in 30% of tumor cells.

Results:
IPT (positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
PR: Intermediate nuclear reaction in 35% tumor cells (Allred score 6/8).
Her2: 30% nuclear reaction of tumor cells.
HER2: No membrane reaction in tumor cells (score 0).
HER3: 30% nuclear reaction of tumor cells.

Technical:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and
Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at.

IMMUNOHISTOCHEMISTRY REPORT

Referred by : mawwa mohamed youssef
Patient Name :
Patient No. :
Path No. :
Sex : Female
Age : 40 Y
Referring Doc. :
Request No. :
Request Date :
Verification Date :
Financial Category :
Department :
IHC No. :
Registered by :
Request Date : 13007569
Verification Date : 12/08/2023
Financial Category : 24/08/2023
Department : Out Patient
Referred by : mawwa mohamed youssef

Presidential Initiative for Women's Health - Pathology Lab
مبادرة الرئيسة لصحة المرأة - مختبر الأمراض



Patient Name:	Naglan Ahmed El-sayed Ahmed	Patient Gender:	Female
Patient Code:	Q38684	DATE:	10/16/2023 9:52:34 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

"HEMROCK" REPORT

Clinical history

- Female with history of left breast cancer, for assessment.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- Increased tracer uptake is seen involving right ankle and shaft of right tibia; site of surgical intervention.
 - Otherwise no abnormal osseous tracer uptake is noted that would account for serious osseous lesions.
 - Degenerative changes of the spine are noted.
- NB: soft tissue uptake at the left breast is noted.

Opinion

- Osseous uptake in right ankle and mid part of right leg attributed to site of known surgical intervention, for radiologic correlation to exclude underlying serious osseous lesions.

Dr. Omnia Alaa

Dr. Mahasen Amin, MD

Patient Name: Naglaa Ahmed Al Sayed Ahmed Khorshid	Date : 09/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID: 3228/2023	Pat ID: 42937

**DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

Indication:

Patient is 41 years old and is seen for follow-up after left MRM

Report:

- Left MRM:
 - Operative bed shows no masses or sizable collections.
 - No regional lymphadenopathy.
 - Mild skin thickening & subcutaneous edema. Consistent with cellulitis.
- Right breast:
 - Scattered fibroglandular density (ACR B)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2), apart from mild skin thickening & subcutaneous edema (consistent with cellulitis).
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr. Eman Abdel Razik, MSc, FEBR
Dr. Amira H. Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
09/07/2024 2:27 PM

Name : Nadia Ahmed El-Sayed Ahmed

Date : 05-01-2026 02:29

Branch : Mamm

Medical No : 219211

Ref: Professor Prof. Gustav Roussy Egypt

Ins Company : Walkan

**DIGITALIZED LOW DOSE SOFT TISSUE MAMMOGRAPHY
WITH COMPLEMENTARY BREAST US EXAMINATION OF
RIGHT BREAST REVEALED:**

- ACR B: Mixed fibro-fatty-glandular breast parenchyma.
- No suspicious irregular opacities or pathological microcalcification clusters.
- No solid or cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- Non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Normal sonomammographic examination of right breast (BIRADS I); for annual follow up.

Much Obligated;

Dr. Heba El Balshy.MD

Lecturer and Consultant of diagnostic and interventional Radiology
NLI-Menofia University

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0 : Needs further investigations.

BIRADS 1: Normal findings.

BIRADS 2: Benign.

BIRADS 3: Probably benign.

BIRADS 4: Probably malignant.

BIRADS 5: Malignant.

BIRADS 6: Proven pathologically malignant

AHMED ELSAYYED, NAGLAA Patient ID: 10110225022268 DOB: 28-Nov-1982 Date: 17-Dec-2025

Patient Lastname: AHMED ELSAYYED

Patient Firstname: NAGLAA

HIS Patient ID: 10110225022268

RIS Patient ID: 379226

Date of Birth: 28/11/1982

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (17/12/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
" Warm radiotracer uptake seen over the distal 2/3 of the right tibial shaft likely post fixation remodeling for clinical correlation.

" Degenerative changes are noted over the cervico-dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton."

Much Obligated

Hoda Yasser

Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====